Review Form of ITCC 2026

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Paper ID:	TC0034	
Paper Title:	Multimodal AI Framework for Personalized and Context-Aware Skin Disease Diagnosis, Monitoring, and Treatment Support	
Topic	**The Topic's Conformity**	☑Match to the conference topic **very well**; □Match to the conference topic **fairly**; □Match to the conference topic **poorly**;
	The Coverage of the Topic	☑Sufficiently comprehensive and balanced □Important Information is missing or superficially treated □Certain parts significantly overstressed
Contents	**Innovation**	□Highly Innovate ☑Sufficiently Innovate □Slightly Innovate □Not Novel
	Integrality	□Poor ☑Fair □Good □Outstanding
	The "literary" presentation	□Totally Accessible ☑Mostly Accessible □Partially Accessible □Inaccessible
	The technical depth	□Superficial □Suitable for the non-specialist ☑Appropriate for the generally knowledgeable individual working in the field □Suitable only for an expert
Presentation & English	□Satisfactory ☑Needs improvement □Poor	
Overall organization	□Satisfactory ☑Could be improved □Poor	

Recommendation for Publication& Detailed Suggestions

Accepted (please choose one)	☐Strongly Accept; ☐Accept; ☑weakly Accept
	Comments Please prepare the final version of the paper as per review instructions: 1. The abstract clearly presents the motivation and highlights the use of a multimodal AI framework; however, it appears overly ambitious and somewhat generic in describing contributions. The claims of improved robustness, fairness, and real-world applicability are not sufficiently supported with concrete numerical evidence or specific experimental settings. The abstract would benefit from including clearer performance indicators, dataset descriptions, and limitations to provide a more balanced and credible summary. At present, it reads more like a proposal than a validated study. 2. The introduction provides a good contextual background, particularly relating to healthcare

	challenges in Sri Lanka, which strengthens the relevance of the study. However, the research gap is not sharply defined. While unimodal limitations are mentioned, the novelty of this work compared to existing multimodal studies is not convincingly articulated. The authors should explicitly state what differentiates their framework from prior multimodal dermatology systems and avoids repeating general AI trends. Additionally, some statements are descriptive but lack supporting citations or critical discussion. 3. The methodology section is structured and detailed, but it lacks clarity in several critical aspects. The integration of multiple components (classification, severity assessment, domain adaptation, and recommendation) is conceptually interesting, yet the workflow is complex and not sufficiently justified in terms of design choices. Important details such as dataset sources, data split strategies, hyperparameter settings, and training protocols are either briefly mentioned or missing. The fusion approach is simplistic (static weighting), and there is limited justification for selecting this method over more advanced techniques. The methodology appears more engineering-oriented rather than scientifically rigorous, and reproducibility is currently weak. 4. The conceptual design is ambitious and integrates several modern AI concepts, which is a strength. However, the framework lacks depth in theoretical grounding and validation. It appears to combine multiple modules without strong evidence of how each contributes independently to performance improvement. The inclusion of rule-based severity scoring and knowledge-graph recommendations seems loosely connected to the core learning model, raising concerns about coherence. The overall quality is moderate, but the work needs stronger conceptual justification and clearer system architecture representation. 5. The results section shows high performance, especially for the image-only model achieving near-perfect accuracy (0.98), which raises concerns about dataset bias or overfitting. The evaluation appears limited to a small set of metrics and lacks robustness checks such as cross-validation, external datasets, or statistical significance analysis. The comparison between models is too simplistic, and no benchmarking against state-of-the-art methods is provided. Additionally, the absence of ablation studies weakens the credibility of claims regarding the contribution of each module.
Rejected (please choose one)	☐ Strongly Reject ☐ Reject ☐ weakly Reject Comments 1. 2. 3. 4. ☐Paper is not of sufficient quality or novelty to be published in the Proceeding ☐A major rewrite is required, encourage resubmission ☐Other comments